

SAME SIZE ARTWORK
LEAFLET SIZE : 170 mm x 225 mm

25 mm

34 mm

SPACE FOR PHARMACODE

the disposition of desloratadine. Grapefruit juice had no effect on the disposition of desloratadine.

PHARMACEUTICAL PARTICULARS

List of excipients:
Microcrystalline Cellulose (Avicel pH 112) Ph. Eur.
Lactose Monohydrate (Tabletose 100) Ph. Eur.
Maize Starch (Unipure-F) Ph. Eur.
Colloidal Silicon Dioxide (Aerosil 200) Ph. Eur.
Magnesium Stearate Ph. Eur.

Incompatibilities
Not Applicable

Shelf life
24 Months

Special precautions for storage
Store below 30°C.
Keep all medicines out of reach of children.

Nature and contents of container
Cold formable Alu/Hard tempered aluminium foil blister containing 10 tablets each. 3 such blisters packed along with leaflet.

Pack Size: 3x10's

DATE OF REVISION OF THE TEXT
Nov 2020

SPACE FOR PHARMACODE

For the use only of a Registered Medical Practitioner
or a Hospital or a Laboratory

GLENDES[®] 5mg Tablets
Desloratadine

Label Claim
Each uncoated tablet contains: Desloratadine Ph. Eur.5 mg

PHARMACEUTICAL FORM
Tablets

Off-white to light pink, circular, Biconvex with 'L5' debossed on one side and plain on the other side.

CLINICAL PARTICULARS

Therapeutic indications
Glendes 5mg Tablets are indicated for the rapid relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge and itching, congestion/stuffiness, as well as ocular itching, tearing and redness.

Glendes 5mg Tablets are also indicated for the relief of symptoms associated with chronic idiopathic urticaria such as the relief of itching and the size and number of hives.

Posology and method of administration
Posology
Adults and adolescents (12 years of age and older):
One Glendes 5 mg tablet once a day regardless of mealtime. For oral use.

Contraindications
Hypersensitivity to the active substance, to any of the excipients, or to loratadine.

Special warnings and precautions for use
In the case of severe renal insufficiency, desloratadine should be used with caution.

Desloratadine should be administered with caution in patients with medical or familial history of seizures, and mainly young children, being more susceptible to develop new seizures under desloratadine treatment. Healthcare providers may consider discontinuing desloratadine in patients who experience a seizure while on treatment.

Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Interaction with other medicinal products and other forms of interaction
No relevant interactions found with desloratadine tablets and co-administration of erythromycin or ketoconazole.

Pediatric population
There is no data available for pediatric population.

Desloratadine taken concomitantly with alcohol did not potentiate the performance impairing effects of alcohol. However, cases of alcohol intolerance and intoxication have been reported. Therefore, caution is recommended if alcohol is taken concomitantly.

Fertility, pregnancy and lactation
Pregnancy
A large amount of data on pregnant women indicate no malformative nor foeto/neonatal toxicity of desloratadine. As a precautionary measure, it is preferable to avoid the use of desloratadine during pregnancy.

Breast-feeding
Desloratadine has been identified in breastfed newborns/infants of treated women. The effect of desloratadine on

SPACE FOR PHARMACODE

Product Registration Holder:
Glenmark Pharmaceuticals
(Malaysia) Sdn Bhd (660397-W)
D-31-02, Menara Suezcap 1,
No. 2, Jalan Kerinchi,
59200 Kuala Lumpur, Malaysia

Manufactured by:
glenmark
Pharmaceuticals Ltd.
B/2, Mahalaxmi Chambers,
22 Bhulabhai Desai Road,
Mumbai - 400 026 (India).
At: Plot No. S 7, Colvale Industrial Estate,
Colvale, Bardez, Goa - 403 513, INDIA
® Registered Trade Mark

MY
PE00000

ICONGRAPHICS CODE:

PANTONE SHADE

PANTONE
BLACK
PROCESS C

Supersedes
Artwork Code

PHARMACODE:

newborns/infants is unknown. A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from desloratadine therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

Fertility

There are no data available on male and female fertility.

Effects on ability to drive and use machines

Desloratadine has no or negligible influence on the ability to drive and use machines. Patients should be informed that most people do not experience drowsiness. Nevertheless, as there is individual variation in response to all medicinal products, it is recommended that patients are advised not to engage in activities requiring mental alertness, such as driving a car or using machines, until they have established their own response to the medicinal product.

Undesirable effects

Summary of the safety profile

At the recommended dose of 5 mg daily, the most frequent of adverse reactions were reported as fatigue, dry mouth and headache.

Pediatric population

In adolescent patients, 12 through 17 years of age, the most common adverse event was headache.

Tabulated list of adverse reactions

The undesirable effects containing desloratadine are listed in the following table. Frequencies are defined as very common, common, uncommon, rare, very rare and not known (cannot be estimated from the available data).

System Organ Class	Frequency	Adverse reactions seen with Desloratadine
Metabolism and nutrition disorders	Not known	Increased appetite
Psychiatric disorders	Very rare Not known	Hallucinations Abnormal behaviour, aggression
Nervous system disorders	Common Very rare	Headache Dizziness, somnolence, insomnia, psychomotor hyperactivity, seizures
Cardiac disorders	Very rare Not known	Tachycardia, palpitations QT prolongation
Gastrointestinal disorders	Common Very rare	Dry mouth Abdominal pain, nausea, vomiting, dyspepsia, diarrhoea
Hepatobiliary disorders	Very rare Not known	Elevations of liver enzymes, increased bilirubin, hepatitis Jaundice
Skin and subcutaneous tissue disorders	Not known	Photosensitivity
Musculoskeletal and connective tissue disorders	Very rare	Myalgia
General disorders and administration site conditions	Common Very rare	Fatigue Hypersensitivity reactions (such as anaphylaxis, angioedema, dyspnoea, pruritus, rash, and urticaria)
	Not known	Asthenia
Investigations	Not Known	Weight increased

Pediatric population

The undesirable effects reported for the product containing desloratadine in paediatric patients with an unknown frequency included QT prolongation, arrhythmia, bradycardia, abnormal behaviour, and aggression.

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product.

Overdose

Treatment

In the event of overdose, consider standard measures to remove unabsorbed active substance. Symptomatic and supportive treatment is recommended.

Desloratadine is not eliminated by haemodialysis; it is not known if it is eliminated by peritoneal dialysis.

Symptoms

Up to 45 mg of desloratadine administration (nine times the clinical dose), no clinically relevant effects reported.

Pediatric population

The adverse event profile associated with overdosage, as seen during post-marketing use, is similar to that seen with therapeutic doses, but the magnitude of the effects can be higher.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

Pharmacotherapeutic group: antihistamines H₁ antagonist, **ATC code:** R06AX27

Mechanism of action

Desloratadine is a non-sedating, long-acting histamine antagonist with selective peripheral H₁-receptor antagonist activity. After oral administration, desloratadine selectively blocks peripheral histamine H₁-receptors because the substance is excluded from entry to the central nervous system.

Desloratadine has demonstrated antiallergic properties. These include inhibiting the release of proinflammatory cytokines such as IL-4, IL-6, IL-8, and IL-13 from human mast cells/ basophils, as well as inhibition of the expression of the adhesion molecule P-selectin on endothelial cells. The clinical relevance of these observations remains to be confirmed.

Pharmacokinetic properties

Absorption

Desloratadine plasma concentrations can be detected within 30 minutes of administration.

Desloratadine is well absorbed with maximum concentration achieved after approximately 3 hours; the terminal phase half-life is approximately 27 hours. The degree of accumulation of desloratadine was consistent with its half-life (approximately 27 hours) and a once daily dosing frequency. The bioavailability of desloratadine was dose proportional over the range of 5 mg to 20 mg.

Distribution

Desloratadine is moderately bound (83% - 87%) to plasma proteins. There is no evidence of clinically relevant medicine accumulation following once daily dosing of desloratadine (5 mg to 20 mg) for 14 days.

Biotransformation

The enzyme responsible for the metabolism of desloratadine has not been identified yet, and therefore, some interactions with other medicinal products cannot be fully excluded. Desloratadine does not inhibit CYP3A4 or CYP2D6 and is neither a substrate nor an inhibitor of P-glycoprotein.

Elimination

There was no effect of food (high-fat, high caloric breakfast) on

PE00000 MY

ICONGRAPHICS CODE:

PANTONE SHADE	
	PANTONE BLACK PROCESS C
Supersedes Artwork Code	

PHARMACODE: